جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : فاطمه جمعه جادالرب عوض
المحافظة : الاسكندريه **الحي / المدينة :** اللبان **العنوان :** ئئ
رقم البطاقة : 26610010105103 **رقم التليفون :** 01148455025 **تاريخ تسجيل الإستمارة :** 10-10-2026
العمر(تاريخ الميلاد) : 01-10-1966 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام رنين مغناطيسي علي الفقرات القطنيه بالصبغه [12771] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: فاطمه جمعه جادالرب عوض
National ID: 26610010105103

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

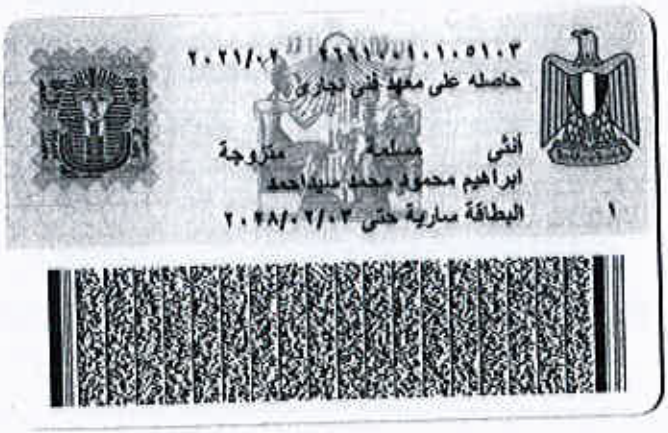
تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced MRI of the lumbar spine

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





National ID - Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

جوستافروسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

الست المصرية هي صحة مصر

MDT FORM 25/1/2026 G

Name	فاطمة محمد طارق	MRN	199/c.cx
		ID	٢٧٦١٠٠١٠٥١٠٣
Age	60 yrs	Phone	01148455025
Menopausal status	postmenopausal	Marital status	01148042386
NRS SCORE		Family history	-ve
BMI		REASON OF MDT	MBC
CO morbidity	-DM		

Clinical Data

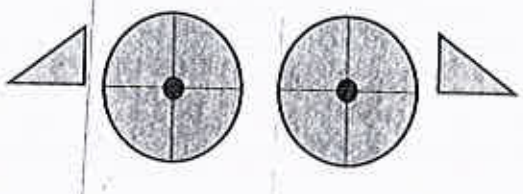
BX: IDC cIII

ER:	PR:	HER 2:	Ki67:
8/8	-ve	-ve	

Sonomammography:

Mets Workup:

R L



TNM

Recommendations:

2021 lt breast Ca → lt MRM → CH → PORT → TC x5yr

2022 lung mets → biopsy ER+ PR-ve HER2-ve → Femara x2 yr → OP → Faslodex/Ibrance (interrupted) x3 → RDP (Brain) → CBCT → Taxol x3 → DP → Xeloda

PR low ↑ Cap + Ibrance → Capecitabine

MDT OUTCOME & PLAN

① US guided Biopsy from HFL

② Femara / Farbo.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

MDT Form
[[DOCTYPE:MDT_FORM]]

Name: فاطمه جمعة جاد **MR. :** 927	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient has metastatic breast cancer with documented metastases to the lung, liver, brain, and bone. The initial regimen (C1) consisted of gemcitabine 1560 mg and carboplatin administered on day 1, with subsequent cycles C2 on 7 March and C3 on 28 March as recorded by pharmacy dispensing.

On day 8 of cycle 1, gemcitabine 1560 mg and carboplatin AUC 2 were prescribed for the next clinic visit.

For cycle 2, day 1, the patient's weight was 69 kg, height 148 cm (BSA 1.68 m²). Gemcitabine 1680 mg and carboplatin AUC 2.5 were administered on day 1 and planned for day 8. On day 8 of cycle 2, gemcitabine 1680 mg and carboplatin AUC 2.5 (270 mg) were dispensed.

On 28 March, the patient reported grade 2 peripheral neuropathy, performance status I–II, and had an unremarkable examination. Cycle 3, day 1 was initiated with gemcitabine 1680 mg and carboplatin AUC 2.5; day 8 dosing was scheduled. Supportive therapy with gabapentin 300 mg daily and duloxetine 30 mg daily was added. Pharmacy records on the same day confirmed weight 69 kg, BSA 1.68 m², and acceptable laboratory values for cycle 3 day 1.

The patient experienced an HSV reaction to IV contrast and required duplex ultrasound for suspected thrombosis. New imaging (CT chest/abdomen/pelvis on 11 April and MRI brain on 11 April) was ordered. Cycle 4, day 1 was planned with gemcitabine and carboplatin AUC 2.5; however, the patient did not receive day 8 dosing on 18 April, and the pharmacy later dispensed gemcitabine 1680 mg and carboplatin AUC 2.5 (240 mg) for the missed dose.

Cycle 5, day 1 was prescribed with gemcitabine and carboplatin, with the next clinic visit scheduled for 7 June and a subsequent cycle 6, day 1 pending new labs. Pharmacy confirmed dispensing of gemcitabine 1680 mg and carboplatin AUC 2.5 on day 1 of cycle 5. The patient missed the prior week's dose, prompting repeat dispensing of the same regimen. On 2 June, cycle 5, day 8 was administered with gemcitabine 1600 mg and carboplatin AUC 2, with labs reviewed and accepted.

Cycle 6, day 1 was administered on 13 June with gemcitabine 1600 mg, carboplatin AUC 2, and supportive therapy (PPI and anti-emetic). Day 8 dosing was planned. On 15 June, platelet count was 62 ×10⁹/L, leading to a one-week postponement and repeat CBC. On 20 June, cycle 6, day 1 gemcitabine dose was reduced by 10% to 1440 mg due to prior grade 2 thrombocytopenia; carboplatin AUC 2 was given along with a blood transfusion.

The oncology history includes left mastectomy in 2004 (stage T1 N2), adjuvant chemotherapy and tamoxifen, recurrence in 2022 treated with letrozole, progression in 2025 with new lung, osseous, and liver metastases treated with faslodex, ibRance, and taxol, followed by gemcitabine/carboplatin

تحريرا في ,

التاريخ :	**20-09-2026**	**الوقت :**	

الطبيب المعالج : **dina makram**

يعتمد ,

initiated in January 2026 (cycle 6, day 8). Imaging on 27 June showed stable disease in liver, regression of mediastinal lymph nodes, stable pulmonary nodule, and new sclerotic focus at L3; brain MRI demonstrated regression of cerebellar lesions.

Cycle 7, day 1 was planned with gemcitabine 1440 mg and carboplatin AUC 1.5, postponed previously due to platelet count of 99 $\times 10^9$/L; vitamin D supplementation was noted. Cycle 7, day 8 was administered with gemcitabine and carboplatin, and CBC results were accepted. CT chest/abdomen/pelvis on 25 July showed stable disease in liver and mediastinal nodes, regression of pulmonary nodule, and a sclerotic L3 focus; MRI brain showed regression of cerebellar lesion. Cycle 7, day 8 gemcitabine and carboplatin were given.

For cycle 8, day 1, gemcitabine 1440 mg, carboplatin AUC 1.5, and prophylactic filgrastim were prescribed after a delay due to thrombocytopenia. Cycle 8, day 8 gemcitabine 1440 mg and carboplatin AUC 1.5 were dispensed, with pending completion of imaging studies. To date, the patient has completed eight cycles of gemcitabine and carboplatin.

Name : fatema gomaa gad al rabb
Date : 15-01-2026 03:35
Branch : Portable
Medical No. : GRI-27497
Ref.Physician Prof :
Ins.Company :

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

As compared to previous study dated 1/11/2025, the current study revealed

1-Chest:

- Left mastectomy with no operative bed enhancing lesion or collection
- Diffuse interstitial pulmonary disease with interstitial thickening and subpleural honey combing
- Stable pulmonary consolidative patch at the anterior segment of left upper lobe
- Still seen bilateral pulmonary nodules, the largest is 7mm.
- Regressive course regarding the size of the previously mentioned enlarged hilar and mediastinal LNS, he largest is retrocaval measuring 15x10mm at the current study compared to 22x17mm at the previous study
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2-Abdomen and pelvis:

- The liver is enlarged in size, Mild progressive course in the size of the previously mentioned bilobar hypodense focal lesions, the largest is yet seen at segment VII measuring 21x23mm compared to 13x13mm seen at segment III at the previous study
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and supra-renal.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Umbilical hernia is noted with omental and bowel loops

3-The scanned skeleton:

- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.
- No vertebral structural collapse or extra-osseous soft tissue component.
- Dorso-lumbar spondylosis.

OPINION:

- Status follow up
- Regressive course as regarding the size of the mediastinal and hilar LNS
- Progressive course as regarding the size of the hepatic focal lesions
- Stationary rest of the study

CT Scan
[[DOCTYPE:CT_SCAN]]

Name : fatema gomaa gad al rabb
Branch : Portable
Ref.Physician Prof :

Date : 15-01-2026 03:35
Medical No. : GRI-27497
Ins.Company :

DR/NERMEEN RAMADAN.MD

DOC GRI

DOC GRI

CT Scan
[[DOCTYPE:CT_SCAN]]

Patient Name: Fatma Gomaa Gad Al Rabb Awad Allah	Date : 01/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 59 Year(s)
Medical ID:	Pat. ID: 27497

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

Compared to the previous study dated 29/05/2025.

1. ***Chest:***

- Left mastectomy with no operative bed enhancing lesion or collection.
- Diffuse intestinal disease with interstitial thickening and honey combing .
- stable left pulmonary consolidative patch at anterior segment of left upper lung lobe.
- Stationary course course of the currently bilateral pulmonary nodules are seen, the largest at left upper lobe measuring 7 mm.
- Regressive course regarding the size and number of enlarged hilar and mediastinal lymph nodes are noted with the largest one at retrocaval group measures 22 x17 mm compared to 3.5 X 2.8 cm.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. ***Abdomen and pelvis:***

- The liver is mildly enlarged with homogenous low CT texture. With multiple newly noted focal lesions , the largest seen at segment III measures 13 x 13 mm.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Mildly enlarged spleen with no focal lesions seen.
- Newly developed few mesenteric lymph nodes reaching 1 cm at right lumbar region.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.

CT Scan
[[DOCTYPE:CT_SCAN]]

Patient Name: Fatma Gomaa Gad Al Rabb Awad Allah	Date : 01/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 59 Year(s)
Medical ID:	Pat. ID: 27497

- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Umbilical hernia is noted with omental and bowel loop content.
- No abdominal or pelvic masses or collections.

3. The scanned skeleton:

- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

- ***Status follow up showing regressive course as regard size of hilar and mediastinal lymph nodes***
- ***Newlynoted multiple bilobar hepatic focal lesions .***
- ***Rather stationary rest***

Signature Reeham.Khader
03/11/2025 5:51 PM

Signature Reeham.Khader
04/11/2025 10:17 AM

CT Scan
[[DOCTYPE:CT_SCAN]]

GUSTAVE ROUSSY
INTERNATIONAL
AGENCY

Patient Name: Fatma Gomaa Gad Al Rabb Awad Allah	Date : 27/09/2025
Exam : MRI BRAIN WITH CONTRAST	Age : 59 Year(s)
Medical ID: 199/2023	Pat. ID: 27497

OPEN MRI EXAMINATION OF THE BRAIN

TECHNIQUE:

Axial T1, T2 and FLAIR WI's
Sagittal T1 WI's
Coronal T2 WIs
Post contrast axial, Sagittal and coronal T1 WIs

FINDINGS:

- Multiple variable sized small space occupying lesions are seen at the right and left cerebellar regions, brainstem as well as a tiny enhancing lesion in the left parietal region. They elicit isointense signal in T1 & T2 WIs with post contrast enhancement with the largest lesion seen in the right cerebellum measuring about 9 x 7 mm at its maximal diameters. Minimal perifocal edema is seen surrounding some of the lesions with minimal mass effect on the adjacent structures.



- Normal size and configuration of the ventricular system.
- No midline shift or deformity.
- Normal cortical sulci and extra axial CSF spaces.
- Intact cervico medullary junction.
- No intra or extra axial areas of fresh blood signal intensities or its degradation products.



OPINION:

No available clinical data showing multiple brain metastatic deposits mainly at the posterior fossa as above described.

Much Obliged;
Dr.Marwa Elgawish, MSc, FEBR

Much Obliged;
Dr.Marwa Elgawish, MSc, FEBR

Much Obliged;
Dr.Marwa Elgawish, MSc, FEBR

M.R.I
[[DOCTYPE:MRI]]

Patient Name: Fatma Gomaa Gad Al Rabb Awad Allah	Date : 16/04/2025
Exam : DUPPLEX ARTERIES BOTH LOWER LIMBS	Age : 58 Year(s)
Medical ID: 199/2023	Pat. ID: 27497

Duplex Examination of the arterial system of both lower limbs revealed: -

Arterial System: **-**

- *Patent arterial system of the examined lower limb including common femoral, superficial femoral, popliteal, anterior and posterior tibial arteries with mild increase in intimal thickness more evident at infrapopliteal vessles.*
- *Preserved arterial triphasic waveform configuration with average flow velocities of the examined arterial tree.*
- *No evidence of arterial stenosis or occlusion seen.*

Signature Dr . Nermien Ramadan
PM 12:39 2025/04/16

Ultrasound
[[DOCTYPE:ULTRASOUND]]

Patient Name: Fatma Gomaa Gad Al Rabb Awad Allah	Date : 27/09/2025
Exam : MRI BRAIN WITH CONTRAST	Age : 59 Year(s)
Medical ID: 199/2023	Pat. ID: 27497

OPEN MRI EXAMINATION OF THE BRAIN

TECHNIQUE:

Axial T1, T2 and FLAIR WI's
Sagittal T1 WI's
Coronal T2 WIs
Post contrast axial, Sagittal and coronal T1 WIs

FINDINGS:

Compared to the study dated 09/07/2025

- progression regarding the size of some of the multiple variable sized small space occupying lesions are seen at the right and left cerebellar regions, brainstem as well as a tiny enhancing lesion in the left parietal region. They elicit isointense signal in T1 & T2 WIs with post contrast enhancement with the largest lesion seen in the right cerebellum measuring about 9 x 7 mm at its maximal along its axial dimensions compared to 5 x 4 mm. Minimal perifocal edema is seen surrounding some of the lesions with minimal mass effect on the adjacent structures.
- Normal size and configuration of the ventricular system.
- No midline shift or deformity.
- Normal cortical sulci and extra axial CSF spaces.
- Intact cervico medullary junction.
- No intra or extra axial areas of fresh blood signal intensities or its degradation products.





OPINION:

No available clinical data showing:

Progression regarding size of some of the multiple brain metastatic deposits mainly at the posterior fossa as above described.

Much Obliged;

M.R.I
[[DOCTYPE:MRI]]

ROUSSY
INTERNATIONAL

Patient Name: Fatma Gomaa Gad Al Rabb Awad Allah	Date : 09/07/2025
Exam : MRI BRAIN WITH CONTRAST	Age : 59 Year(s)
Medical ID: 199/2023	Pat. ID: 27497

MRI EXAMINATION OF THE BRAIN

- **Peripheral b rain involutional changes in the form of prominenet extra axial CSF spaces.**
- **Three SOLs are noted both cerebellar hemispheres eliciting high T2/FLAIR signal intensity with isointense signal in T1 with homogenous contrast enhancement , two are seen at left cerebellar hemisphere measuring 2 and 3mm, the largest one seen at right cerebellar hemisphere5 x4 mm.no surrounding edema .**
- **Two ovoid shape lesion are seen at calcvarial spaceat both occipital bones , eliciting high T1/T2 signal likely of fat signal .**
- Normal size and configuration of the ventricular system with no evidence of deformity or displacement.
- No mid line shift.
- Normal appearance of both cerebral hemispheres.
- No brain stem parenchymal areas of abnormal signal intensity.

OPINION:

- *Three cerebellar SOL as described likely metastatic .*
- *Two calvarial occipital lesions for chemical shift sequences.*

DR/Reham Khedr,,MD

M.R.I
[[DOCTYPE:MRI]]





National ID - Front & Back
[[DOCTYPE:ID_FRONT_BACK]]